Chapman & DeLapp (2013) presented a detailed case study of treating an adult male with *blood-injection-injury (BII) phobia* using a nine-session manualized cognitive-behavioral therapy (CBT) program combined with applied tension techniques. The study highlights how structured CBT can effectively reduce phobic symptoms and improve functioning.

📌 Case Summary

- **Patient**: Adult male with severe blood-injection-injury phobia.
- **Treatment**: Nine-session manualized CBT program.
- **Techniques Used**:
  - **Applied tension**: Teaching the patient to tense muscles to counteract fainting (common in BII phobia).
  - **Exposure therapy**: Gradual exposure to feared stimuli (needles, blood).
  - **Cognitive restructuring**: Challenging irrational beliefs about medical procedures and blood.
- **Outcome**: Significant reduction in phobic symptoms, improved tolerance of medical procedures, and decreased avoidance behaviors.

🔑 Key Points

- **Aim of Study**: To explore "active mechanisms of change" in CBT for BII phobia, focusing on how applied tension and cognitive restructuring work together.
- **Applied Tension Effectiveness**: Prevented fainting episodes, which are a hallmark of BII phobia.
- **Manualized CBT**: Structured, replicable approach that can be generalized to other patients with similar phobias.

- **Mechanisms of Change**:
  - Reduction of catastrophic thinking.
  - Increased sense of control during exposure.
  - Physiological regulation through applied tension.
- **Clinical Significance**: Demonstrates that short-term, structured CBT can be highly effective for specific phobias, especially those with strong physiological components like fainting.
- **Broader Implications**: Supports CBT as a frontline treatment for fear-related disorders and emphasizes the importance of tailoring techniques (like applied tension) to the specific phobia type.

📊 Summary Table

Aspect	Details
Phobia Type	Blood-Injection-Injury (BII)
Treatment Length	9 sessions
Core Techniques	Applied tension, exposure therapy, cognitive restructuring
Outcome	Reduced phobic symptoms, improved tolerance of medical procedures
Key Contribution	Demonstrated active mechanisms of change in CBT for BII phobia

⚠️ Considerations & Limitations

- **Single Case Study**: Results cannot be generalized without caution.
- **Manualized Approach**: While structured, individual differences may require adaptation.

- **Ethical Note**: Exposure therapy must be carefully managed to avoid overwhelming patients.

👉 In short, Chapman & DeLapp (2013) is a landmark case study showing how **applied tension plus CBT can successfully treat blood-injection-injury phobia** in a structured, short-term format.